

VALUE ASSESSMENT

Brad Larmie

Exploring how ambient documentation could create value for your outpatient practice—with transparent methodology you can stress-test.

+\$1.01M

NET ANNUAL VALUE

6.6x

RETURN ON INVESTMENT

3,675

HOURS RETURNED

EXECUTIVE SUMMARY

Your Value Model at a Glance

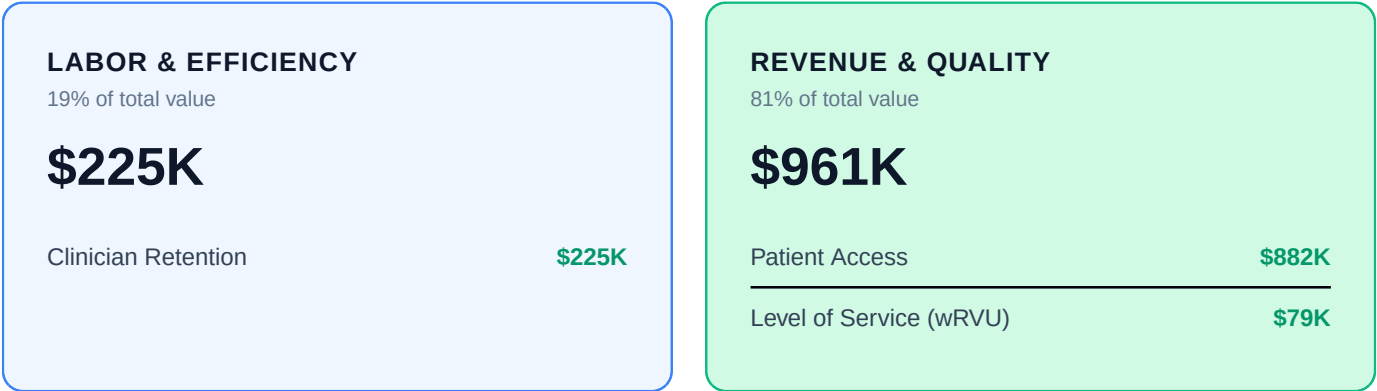
60 providers • 126,000 annual encounters • 3 value drivers

How to use this assessment

This model translates your inputs into projected value using industry benchmarks and conservative assumptions. Every number traces back to editable inputs—designed so you can stress-test the assumptions and adjust where your situation differs.



Where the Value Comes From



Multi-Year Projection

	Year 1	Year 2	Year 3	3-Year Total
Value	\$1.19M	\$1.31M	\$1.44M	\$3.93M
Investment	\$180K	\$180K	\$180K	\$540K
Net Value	\$1.01M	\$1.13M	\$1.26M	\$3.39M

Patient Access

When clinicians spend less time on documentation, they may have capacity for additional patient visits. The conversion isn't automatic—scheduling, room...

ANNUAL VALUE
\$882K

THE LOGIC

Time saved per encounter multiplied by eligible encounters gives total hours returned. We allocate a percentage to patient access, then apply a conversion rate to account for real-world scheduling constraints. The result represents realistic capacity expansion—not theoretical maximum.

YOUR CALCULATION

STEP 1: TIME RETURNED

How much documentation time does Abridge return?

88,200

encounters

×

2.5 min

=

3,675 hours

Based on your time savings scenario selection.

STEP 2: HOURS TO ACCESS

How much time can realistically convert to patient access?

3,675

hours

×

25%

allocation

=

0 hours

Not all time converts—some goes to wellbeing, teaching, research.

STEP 3: VISIT CONVERSION

How many hours actually become visits?

0

hours

×

50%

realization

=

0 visits

Realization rate accounts for scheduling, room availability, demand.

STEP 4: ANNUAL VALUE

What's the revenue impact?

0

visits

×

\$200

per visit

=

\$882K

Annual Patient Access Value
0 visits × \$200/visit

\$882K

WHAT THIS MEANS

At \$882K annually, patient access represents 74% of your projected value. Organizations with strong scheduling operations and patient demand often see this number grow as utilization matures.

Clinician Retention

Documentation burden is consistently cited as the leading driver of physician burnout. Replacing a departing clinician costs \$400K-\$1M+ when factoring...

ANNUAL VALUE
\$225K

THE LOGIC

Starting with your baseline turnover rate, we estimate what portion relates to burnout and how much documentation contributes. A conservative retention improvement rate reflects that while ambient AI helps significantly, it doesn't eliminate all burnout drivers.

YOUR CALCULATION

STEP 1: EXPECTED TURNOVER

How many departures occur annually?

60

providers

×

7%

turnover

=

0.0 departures

STEP 2: BURNOUT-RELATED

How many are tied to burnout?

0.0

departures

×

50%

=

0.00 burnout-related

STEP 3: PREVENTED DEPARTURES

How many can improved documentation help prevent?

0.00

at-risk

×

30%

impact

=

0.00 prevented

Conservative—documentation is a major driver but not the only one.

STEP 4: RETENTION VALUE

What's the cost savings?

0.00

prevented

×

\$400K

cost

=

\$225K

Annual Clinician Retention Value

0.00 prevented × \$400K

\$225K

WHAT THIS MEANS

Retention value is probabilistic—it materializes over 12+ months as turnover patterns emerge. With 60 providers, even fractional improvement in retention creates substantial value given replacement costs.

Level of Service (wRVU)

The hypothesis behind wRVU improvement is that physicians under time pressure often document less than the full clinical picture. When notes capture t...

ANNUAL VALUE

\$79K

THE LOGIC

We calculate baseline wRVUs from your encounter volume, then apply a modest improvement percentage based on better documentation. The Medicare conversion factor provides a conservative baseline—commercial rates would yield higher results. A realization rate accounts for payer mix and fee schedule variability.

YOUR CALCULATION

STEP 1: BASELINE WRVUS

What's your current wRVU generation?

88,200

encounters ×

1.5

wRVU/enc =

0 wRVUs

STEP 2: DOCUMENTATION LIFT

How much improvement does better documentation create?

0

wRVUs ×

5%

lift =

0 wRVU gain

Lift comes from capturing complexity that supports accurate coding.

STEP 3: DOLLAR VALUE

What's the financial impact?

0

wRVUs ×

\$33

CF ×

75%

realization =

\$79K

Medicare CF shown. Commercial rates (\$45-65) yield higher results.

Annual Level of Service (wRVU) Value

0 wRVU × \$33 × 75%

\$79K

WHAT THIS MEANS

This \$79K represents documentation improvement value—capturing complexity that's already being delivered. It's highly defensible because it's not about doing more, it's about getting credit for existing work.

SCALING JOURNEY

From Pilot to Full Deployment

Exploring how value compounds as adoption matures—and where the math might differ for your organization.

The compounding effect

Value doesn't scale linearly. As utilization improves (70% '85%) and workflows adapt, each provider may generate more value at maturity than at pilot. These projections assume adoption patterns we've observed—your experience could differ.



STARTING POINT

- 60 providers
- 126,000 encounters
- 70% utilization
- \$1.19M /year

FULL SCALE POTENTIAL

- 180 providers
- 85% utilization
- Scaling: 24 months
- \$5.19M /year

VALUE MULTIPLIER

4.4x

From pilot to full scale, projected value grows from \$1.19M to \$5.19M. This includes \$1.63M in compounding effects from workflow maturity and utilization improvement.

METHODOLOGY

How We Built This Model

Understanding the inputs, benchmarks, and principles behind these projections.

Our approach

This model prioritizes transparency over precision. We use conservative assumptions, show our work step-by-step, and make every input editable. The goal isn't to prove a number—it's to give you a framework for thinking about value that you can stress-test and adapt.

YOUR INPUTS

- 60 providers
- 126,000 annual encounters
- 70% utilization rate
- 2.5 min saved per encounter
- \$250/mo per provider

ENABLED DRIVERS

- Patient Access: \$882K
- Clinician Retention: \$225K
- Level of Service (wRVU): \$79K

KEY PRINCIPLES

- Conservative realization rates
- Medicare CF for wRVU (not commercial)
- Audit factors for risk adjustment
- Probabilistic retention modeling
- All assumptions editable

REALIZATION RATES EXPLAINED

Not every hour saved creates a dollar. Scheduling constraints, minimum shift requirements, payer mix, and other real-world factors mean only a portion of theoretical value converts to actual value. Each driver includes a realization rate—a conservative discount that reflects these constraints. You can adjust these rates to match your organization's reality.

Investment Summary

Annual Investment	\$180K
Projected Annual Value	\$1.19M
Net Annual Value	\$1.01M

This assessment is for strategic planning purposes. Actual results will vary based on implementation, adoption, and organizational factors. We recommend validating key assumptions with your finance and operations teams before using these projections for budgeting or business cases.